Safeguarding and PSEA Policy

1 Purpose and scope

This policy sets out how Girl Pride Africa Kenya (GPAK) protects children and adults at risk from harm, abuse, neglect and sexual exploitation and abuse (SEA), and what everyone connected to GPAK must do to prevent harm, recognise it and respond to it. It applies across all of GPAK's programmes and the counties where GPAK works.

It applies to all GPAK staff, the Advisory Committee, volunteers, mentor mothers, community volunteers, interns, consultants, contractors, visitors and anyone else acting on GPAK's behalf or present at a GPAK activity. It covers safeguarding of participants and their children, of other children and adults at risk encountered through GPAK's work, and PSEA (protection from sexual exploitation and abuse) in relation to anyone GPAK works with, including community members who are not participants.

This policy is built on the Keeping Children Safe international child safeguarding standards: Policy, People, Procedures and Accountability. Sections 1 to 5 set out the policy; sections 6, 7 and 17 cover people; sections 8 to 16 and 18 set out procedures; section 19 and the Advisory Committee's role in section 6 provide accountability.

2 GPAK's context

GPAK works with adolescent girls and young women aged 13 to 23, many of whom are under 18, across Homa Bay, Siaya, Kisumu and Migori counties. Participants include pregnant girls and young mothers, and may be living with HIV, caring for infants or young children, or recovering from violence. Some are cared for by family members, some by themselves. GPAK delivers support through five programme areas: psychosocial support and mental health; health and HIV referrals; education and school re-entry; livelihoods and agriculture; and family strengthening and parenting.

GPAK's community workforce, mentor mothers and community volunteers, includes many programme graduates: young women who understand participants' circumstances because they have lived through similar ones. This closeness is a strength for engagement and trust, and it is also a safeguarding responsibility: GPAK must be clear about what these roles may and may not do, and must protect both participants and community roles from situations that could lead to harm or to an allegation.

Because participants' circumstances (HIV status, pregnancy, experience of violence) are sensitive, safeguarding in GPAK's work is inseparable from confidentiality. A visit, a phone call or a comment that reveals a participant's status to a neighbour, a relative or a partner is itself a safeguarding risk, not only a data protection issue. This policy treats confidentiality breaches connected to HIV or pregnancy status as a form of harm to be prevented and reported, alongside physical, sexual and emotional abuse.

3 Definitions

- **Child**: any person under 18 years, in line with the Children Act, No. 29 of 2022.
- **Adult at risk**: an adult who, because of her situation (for example a young mother, a person living with HIV, a person with a disability, or a person who has experienced violence) may have a reduced ability to protect herself from harm, abuse or exploitation.
- **Abuse**: physical abuse, emotional or psychological abuse, sexual abuse, neglect and exploitation, whether a single act or a pattern of behaviour, and whether by a stranger, a family member, a peer, a community member or someone connected to GPAK.
- **Sexual exploitation and abuse (SEA)**: any actual or attempted abuse of a position of vulnerability, differential power or trust for sexual purposes, as set out in the IASC principles in section 4. It includes exchanging assistance, money, employment, goods or services for sex or sexual favours.

- **Safeguarding concern**: any worry, observation, allegation or disclosure that a child or adult at risk has been, is being, or might be harmed, abused, neglected or exploited, by anyone: a staff member, a peer, a family member, a community member or a stranger.
- **Disclosure**: when a child or adult at risk tells someone, directly or indirectly, that she has experienced or is experiencing abuse, exploitation, neglect or violence.

4 The IASC Six Core Principles relating to sexual exploitation and abuse

GPAK adopts the Inter-Agency Standing Committee (IASC) Six Core Principles on SEA (2019) in full.

1. Sexual exploitation and abuse by humanitarian workers constitute acts of gross misconduct and are therefore grounds for termination of employment.
2. Sexual activity with children (persons under the age of 18) is prohibited regardless of the age of majority or age of consent locally. Mistaken belief regarding the age of a child is not a defence.
3. Exchange of money, employment, goods, or services for sex, including sexual favours or other forms of humiliating, degrading or exploitative behaviour, is prohibited. This includes exchange of assistance that is due to beneficiaries.
4. Any sexual relationship between those providing humanitarian assistance and protection and a person benefitting from such humanitarian assistance and protection that involves improper use of rank or position is prohibited.
5. Everyone connected to GPAK, staff, the Advisory Committee, mentor mothers, community volunteers and partners, must report any concern or suspicion of sexual exploitation or abuse through GPAK's safeguarding reporting channels, described in section 11.
6. Managers at every level of GPAK share responsibility for creating and maintaining an environment that prevents sexual exploitation and abuse and for putting this policy and the Code of Conduct (POL-02) into practice.

5 GPAK's commitments

- GPAK has zero tolerance for abuse, exploitation and SEA by anyone connected to it.
- Every participant's safety and dignity come before GPAK's reputation, funding or relationships with partners.
- GPAK will not knowingly place a participant at greater risk through its own programming, including through how a home visit, a group session or a referral is carried out.
- GPAK will vet, train and supervise everyone in contact with participants (section 7).
- GPAK will respond to every safeguarding concern promptly, following the procedure in section 11, and will support survivors on their own terms (section 14).
- GPAK will keep the identity of a child or adult at risk who raises a concern confidential, except where sharing information is necessary to protect her or another child, or is required by law.
- GPAK will not investigate a concern using the same person who received it, and concerns about the Director go to the Chair of the Advisory Committee, not to the Director.

6 Roles and responsibilities

Role	Safeguarding responsibilities
Director	Overall accountability for safeguarding; appoints the Safeguarding Focal Point (SFP) and Deputy SFP in writing (Annex S2); decides on suspension and cooperation with authorities for concerns about staff or peers other than the Director; notifies any funder whose programme is affected of serious incidents; reports quarterly to the Advisory Committee.

Role	Safeguarding responsibilities
Safeguarding Focal Point (SFP)	First point of contact for any concern or disclosure; carries out the safety assessment and decides on referral within 24 hours; makes or arranges statutory reports; keeps the restricted safeguarding log (MEL-03); coordinates survivor support; reports to the Director, and to the Chair of the Advisory Committee where a concern involves the Director.
Deputy Safeguarding Focal Point	Carries out the SFP's responsibilities when the SFP is unavailable or is herself part of a concern; receives the same training as the SFP.
Advisory Committee	Oversees safeguarding as part of its governance role; receives the Director's quarterly report; the Chair receives and acts on any concern involving the Director; approves this policy and reviews it annually.
All staff	Complete safeguarding training and sign the Code of Conduct (POL-02) before first contact with a participant; follow the safe programming rules in section 8; report any concern to the SFP within 24 hours, immediately wherever possible.
Mentor mothers and community volunteers	Follow the same conduct standards as staff; identify and pass on concerns immediately; support participants to reach services; keep confidentiality. They must never investigate a disclosure, decide whether it is true, confront the person a concern is about, promise confidentiality, or handle the matter themselves instead of reporting it.

7 Safe recruitment

Everyone in contact with participants, whether staff, a volunteer, a mentor mother or a community volunteer, is vetted and trained before first contact, in line with the Human Resources and Volunteer Policy (POL-10): a national ID check, two references who are not relatives, a Certificate of Good Conduct from the Directorate of Criminal Investigations, a signed Code of Conduct (POL-02), and safeguarding and confidentiality training. Anyone in a community role must be 18 or over. A gap in any of these steps means the person does not have contact with participants until it is closed.

8 Safe programming

8.1 Home visits

- Home visits are scheduled with the participant in advance and logged (date, time, purpose, who is visiting) before the visit takes place.
- Home visits happen only with the participant's consent. GPAK never visits a participant's home unannounced.
- For a visit to a participant under 18, GPAK prefers a public or semi-public setting (for example a facility, a group venue or another agreed neutral place) or, where a home visit is necessary, two GPAK-connected people attend together rather than one person alone.
- The mentor mother or community volunteer records the visit in the follow-up notes described in the Participant Support Standard Operating Procedures (PRG-01).

8.2 Group sessions

Group sessions are organised by county, age band or interest, never by HIV status or any other clinical characteristic. No group activity, invitation or communication reveals who in the group is living with HIV, pregnant, or receiving intensive support.

8.3 Transport and escorted referrals

No one connected to GPAK transports a participant alone in private transport, except an escorted referral that has been logged in advance in the referral and verification records (PRG-02, form F8), at the participant's own request, and agreed with the Programs Officer beforehand.

8.4 Contact, phones and social media

- Contact with participants for GPAK business uses GPAK phones and GPAK airtime, never a personal number given or received for that purpose.
- No one connected to GPAK contacts a participant on personal social media, or accepts or sends a friend or follow request to or from a participant.
- SMS reminders to participants never reveal HIV status or pregnancy, in line with the wording rules in the Participant Support Standard Operating Procedures (PRG-01).

8.5 WhatsApp peer circles

GPAK's WhatsApp peer circles are created and moderated from a GPAK phone by a trained staff member or mentor mother, never a personal number. A participant joins a peer circle by her own consent and may leave at any time without losing any other support. Group names never refer to HIV, pregnancy or any health condition, and no member shares another member's health status, photographs, location or personal story in the group. No one gives clinical advice in a group. The moderator removes a breaching message promptly and reports a serious breach to the Safeguarding Focal Point.

8.6 Counselling sessions

Counselling is held in a setting private enough for a participant to speak freely, but visible enough for a colleague to see that a session is taking place, for example a room with a window or an open door. Only a trained counsellor provides counselling. A participant whose needs go beyond what a trained counsellor can safely address is referred promptly to a qualified mental health professional for clinical care.

8.7 Menstrual health support

Menstrual health education and sanitary pad distribution are carried out discreetly, so that no participant is identified to others, including other participants, as receiving this support.

8.8 Family mediation

Family mediation is offered only where the participant herself wants it and a safety assessment shows it will not put her at greater risk. GPAK never pressures a participant to accept mediation with a family member she does not want involved, and mediation stops immediately if she asks it to.

8.9 School visits

A school visit is arranged with the school in advance, for an agreed purpose such as re-entry negotiation or attendance follow-up. GPAK never discusses a participant's HIV status, pregnancy or other sensitive information with school staff without her specific consent.

9 Conduct standards from girls' own experience

Through GPAK's consultations with girls, participants described specific things that make follow-up feel unsafe or unwelcome, rather than helpful. GPAK treats these as binding conduct standards, not general good intentions.

- **No unannounced visits.** A mentor mother or community volunteer arriving without notice, even with good intentions, is a conduct breach.
- **No sharing of information.** What a participant tells GPAK is not repeated to family, neighbours or other participants without her consent, other than through the safeguarding and statutory reporting routes in this policy.
- **No visits that draw a neighbour's attention.** Follow-up is planned so that the manner, timing, frequency or visibility of contact does not mark a participant out to people around her.
- **No controlling manner.** Follow-up is supportive, not directive. A mentor mother or community volunteer does not pressure, lecture or make a participant feel checked up on or judged.

10 Responding to a disclosure

Anyone who receives a disclosure, from a participant, another child, or an adult at risk, must:

1. Listen calmly, take her seriously, and let her speak in her own way and her own time.
2. Never promise secrecy. Explain, gently, that the information will be shared only with people who need to know in order to help her.
3. Not investigate: do not question her repeatedly, do not ask her to repeat her account to other people, and do not confront the person she is describing.
4. Record what she said in her own words, as soon as possible afterwards, on the Safeguarding concern report form (Annex S1).
5. Report to the Safeguarding Focal Point within 24 hours, immediately wherever possible, and arrange emergency help first (police or a health facility) if she is in immediate danger.

11 Step-by-step reporting and response procedure

Step	Who	By when
1. Notice a concern or receive a disclosure	Any staff member, mentor mother, community volunteer or visitor	Immediately
2. Arrange emergency help if there is immediate danger (police or a health facility)	The person who receives the concern	Before anything else
3. Report to the Safeguarding Focal Point, or the Deputy if the SFP is unavailable or involved	The person who receives the concern	Within 24 hours, immediately wherever possible
4. Complete the Safeguarding concern report form (S1)	The person who receives the concern, with the SFP	Within 24 hours
5. Carry out a safety assessment and decide on referral	Safeguarding Focal Point	Within 24 hours of the report
6. Make a statutory report where the law requires it (Sub-County Children's Officer or police gender desk)	Safeguarding Focal Point	As soon as the assessment is complete, without delay
7. Arrange referral to health, psychosocial or legal support	Safeguarding Focal Point	Within 24 hours of the decision
8. If the concern involves the Director, report to the Chair of the Advisory Committee instead of the Director	The person who receives the concern	Immediately
9. Notify any funder whose programme is affected of a serious incident, without identifying details	Director	Within 48 hours of GPAK becoming aware, or sooner if a grant agreement requires
10. Review and close the case	Safeguarding Focal Point and Director	Once support and any investigation are complete

12 Statutory reporting and confidentiality for minors

GPAK keeps what a participant tells it confidential, with one limit: where a child is at risk of harm, Kenyan law requires GPAK to report to the relevant authority (the Sub-County Children's Officer or the police gender desk), even without the child's agreement. GPAK explains this limit to every participant in plain language at enrolment, before she discloses anything: that what she shares is kept private among the people who need to know to help her, and that GPAK must report to protect her, or another child, if she or another child is at risk of serious harm.

This is explained as a safety measure for her, not as a threat, and it does not change GPAK's commitment to keep information away from family, neighbours and the wider community wherever the law allows.

13 Concerns about a staff member, mentor mother or community volunteer

- The person is suspended from contact with participants pending review; this is a protective step, not a finding of guilt.
- The review is carried out by someone independent of the case: not the person who received the original concern, and not a direct peer of the person concerned. Where the concern involves the Director, the Chair of the Advisory Committee leads or appoints the review.
- GPAK cooperates fully with the police, the Sub-County Children's Officer and any other statutory authority.
- The outcome and any action taken are recorded in the restricted safeguarding log (MEL-03) and reported to the Director and the Advisory Committee.

14 Support to survivors

Support is survivor-centred: led by what she wants and needs, at her pace, and never conditional on cooperating with an investigation. GPAK refers to:

- Health services at a partner or nearby facility, for medical care and, where relevant, post-exposure prevention;
- Psychosocial support, through GPAK's own psychosocial and mental-health support programme or a referral partner;
- Legal support and reporting, through the police gender desk or the Sub-County Children's Officer;
- The National Child Helpline, **116** (free, 24/7);
- The GBV helpline, **1195**.

A survivor's continued access to GPAK's other support (health referrals, education, livelihood or family support) is never reduced or made conditional because she has raised a concern or been part of a safeguarding response.

15 Notifying funders

The Director notifies any funder whose programme is affected of a serious safeguarding incident within 48 hours of GPAK becoming aware, or sooner if a grant agreement requires. The notification describes what happened and what GPAK is doing, without identifying the child, adult at risk, or, unless already publicly known, the person a concern is about.

16 Record keeping

Every safeguarding concern, disclosure and response is recorded in the restricted safeguarding log held in MEL-03. Access to this log is limited to the Safeguarding Focal Point and the Director. Completed Safeguarding concern report forms (Annex S1) are kept in the same restricted, locked storage, separate from routine participant files, for the retention period set out in GPAK's records retention rules.

17 Training

Everyone in contact with participants completes safeguarding and PSEA training, including this policy, the Code of Conduct (POL-02) and how to recognise and report a concern, before their first contact with a participant. All staff, mentor mothers and community volunteers complete an annual refresher. The Safeguarding Focal Point and Deputy receive additional training in receiving disclosures, safety assessment and case handling.

18 Visitors and partners

Visitors, including a funder's representatives, follow the Visitor Briefing and Confidentiality Undertaking (DD-02) before any contact with participants. This covers safeguarding conduct, photography rules and the limits of what a visitor may ask or be told. A visitor never has unsupervised contact with a participant under 18.

19 Monitoring and review

The Safeguarding Focal Point reports safeguarding activity, without identifying details, to the Director monthly and to the Advisory Committee quarterly. This policy is reviewed annually by the Advisory Committee, or sooner if the law changes, a funder requirement changes, or a case review shows that a change is needed.

Annex: Reporting flowchart

Step	Who	By when
Notice a concern or receive a disclosure	Any staff member, mentor mother, community volunteer or visitor	Immediately
Arrange emergency help if in immediate danger	The person who receives the concern	Before anything else
Report to the Safeguarding Focal Point (or Deputy)	The person who receives the concern	Within 24 hours, immediately wherever possible
Complete the Safeguarding concern report form (S1)	The person who receives the concern, with the SFP	Within 24 hours
Safety assessment and referral decision	Safeguarding Focal Point	Within 24 hours of the report
Statutory report if the law requires it	Safeguarding Focal Point	Without delay once the assessment is complete
Referral to health, psychosocial or legal support	Safeguarding Focal Point	Within 24 hours of the decision
Concerns about the Director go to the Chair of the Advisory Committee	The person who receives the concern	Immediately
Notify any funder whose programme is affected of a serious incident	Director	Within 48 hours of GPAK becoming aware, or sooner if a grant agreement requires
Review and close the case	Safeguarding Focal Point and Director	Once support and any investigation are complete

Approval and review

Item	Detail
Policy owner	Director
Approved by	Advisory Committee
Applies to	All GPAK staff, Advisory Committee members, volunteers, mentor mothers, community volunteers, interns, consultants, contractors and partners working on GPAK activities
Review	Annually, or sooner if the law, a funder requirement or a case review shows that a change is needed.

Public copy. Internal forms, annexes and signature pages are kept on file by GPAK and are available on request.